

اسم المريض رابعياً : ساميه بخيت حبشى ابراهيم

المحافظة : المنيا **الحى / المدينة : بنى مزار**

العنوان : /

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 27307142401983 رقم التلفون : 01151972679

العمر(تاريخ الميلاد) : 14-07-1973 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : أبحاث أورام موجات صوتيه علي الثدي والابطين والرقبه [4142]

[4142]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: ساميه بخيت حبشي ابراهيم National ID: 27307142401983	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Ultrasound of the breast, axillae and neck
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني

ب. ح. ع.
9-20



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
صحة
مصر

09/20/2026

MDT FORM

Name: سامية بخيت حشيش ابراهيم	MRN: 11357
Age: 53 Y	ID: 27307142401983
Menopausal Sta	Phone: 01158674135
NRS SCORE:	Marital Status: widow
BMI:	Family History:
Gender:	Reason Of MDT: POST OP
CO Morbidity: blood disease ???	ECOG Performance Status 0 1 2 3 4
Surgical History: hystrectomy	

Clinical Data:

BX: right breast: -Invasive tubular and cribriform carcinoma
ER: 8/8 **PR:** 8/8 **HER2:** LOW **KI67:** 10

History:

Sonomammography:

Mammo

Right breast (BIRADS 5): 10-11 o'clock suspicious mass lesion. Left breast (BIRADS 2): LIQ dermal lesion with surrounding inflammatory changes mostly sebaceous cyst.

Biomarker/Molecular

Mets Workup:

Ct;

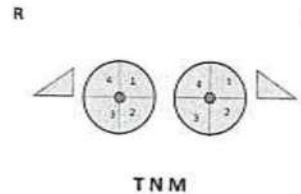
A known case of right breast cancer and the current study shows: Bilateral variable sized axillary lymphadenopathy, larger on the right side. Hyperinflated both lungs, with a thin linear atelectatic band in the posterior segment of right lower pulmonary lobe and mildly thickened the pleura on the right side. Bilateral variable sized axillary lymphadenopathy, larger on the right side. Hepatomegaly with fatty infiltration. Low lying malrotated right kidney, being located in the right lower lumbar region. An irregular mucosal thickening of the gastric wall, denoting a gastritis. No evidences of remote metastatic lesions seen in the present study. For clinical check up.

Recommendations:

ptn underwent upfront rt WLE+SLNB
 Free margins
 SLNB :0/7
 T1b,N0
 FOR adjuvant ttt

MDT Outcome & Plan

for u/s contralat axilla
 if free for adj TAM 5 years
 PORT



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عزت			

جمهورية مصر العربية
بطاقة تحقيق الشخصية



مساميه

بختيت حبشي ابراهيم

١٢ شارع على اغا - عزبة قواطين

مركز بنى مزار - المنيا

١٩٧٣/٠٧

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LV7486092

د. محمد

الحج
كاس
عليه



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٢٧٣.٧١٤٢٤.١٩٨٣

فلاحه

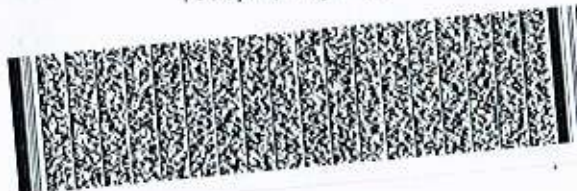


ارملة

مسيحية

أنثى

البطاقة مصرية حتى ٢٠٢٢/٠٧/٠٧



Patient Name	ساميه بخيت حبشي ابراهيم مبادره	Code Number	YK 262870
Sex/Age	Female 52Y	Patient ID	20082
Registered Date	23/05/2026 11:53:34	Referred By Dr.	Interventional Radiology
Validation Date	01/06/2026 15:17:44	Branch	Gustave Roussy
Reporting Date	6/7/2026, 5:49:24 AM		

BREAST and BIOMARKERS

Clinical Data: Right breast mass (BIRADS 5) .

Nature of Specimen: Tru-cut biopsy, collected on 23/05/2026, at 1:10 pm. Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive tubular and cribriform carcinoma
- Histologic grade: 1 (differentiation 1, pleomorphism 2, mitosis 2)
- DCIS: Absent
- Microcalcification: Absent
- Angioinvasion: Absent
- Perineural invasion: Absent

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive tubular and cribriform carcinoma, Grade 1.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Low (score 1+).

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within

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> 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

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Signature
Prof. Dr. Maha Guimei
Dr. Walaa Ibrahim Ghoneim



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 15580 / 0221277647

Name: Mrs. Samia Bekhiet Habashey
Date: 2026-05-06

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 54-year-old female with negative family history, complaining of left breast infection.

Findings:

Right breast.

- Almost entirely fatty breast parenchyma. (ACR a)
- UOQ, middle third, irregular small speculated mass lesion with surrounding distortion, no suspicious microcalcifications.
- **Ultrasound** demonstrated 10-11 o'clock, middle third, 5 cm from nipple, 12 mm deep to the skin, irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring 6 x 6 mm.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs. No pathological infra or supra-clavicular lymph nodes.

Left breast.

- Almost entirely fatty breast parenchyma. (ACR a)
- No masses, speculations, asymmetric density or suspicious microcalcifications.
- **Ultrasound** demonstrated No solid or cystic lesions.
- 7 o'clock middle third dermal lesion measuring 23 x 6 mm with adjacent minimal inflammatory changes.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): 10-11 o'clock suspicious mass lesion.

Left breast (BIRADS 2): LIQ dermal lesion with surrounding inflammatory changes mostly sebaceous cyst.

Recommendation:-

For biopsy and histopathological correlation.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Patient Name	ساميه بخيت حبشي ابراهيم مبادره	Code Number	YK 262870
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Validation Date	01/6/2026 03:17:44 PM	Branch	Gustave Roussy
Reporting Date	06/6/2026 10:43:51 AM	Pathology No.	1638/26

Clinical Data: Right breast mass (BIRADS 5) .

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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%;

Patient's Name: Samia Bakheet Habashi
Date: 2026-06-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Hyperinflated both lungs.
- A thin linear atelectatic band is seen in the posterior segment of right lower pulmonary lobe.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidation or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- A subtle right pleural thickening is seen.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Intact bony thoracic cage.
- Cuts taken in the axillae revealed bilateral variable sized tiny lymph nodes with the largest one measures about 15x7.8mm on the right side.
- Increased sized liver,measuring about 22 cm, showing diffuse reduced parenchymal attenuation with homogenous enhancement pattern.
- No sizable abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys,(with low lying malrotated right kidney in the right lower lumber region) , both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- An irregular mucosal thickening of the gastric wall=13 mm.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvis is unremarkable.

IMPRESSIONS:

A known case of right breast cancer and the current study shows:

- Bilateral variable sized axillary lymphadenopathy,larger on the right side.
- Hyperinflated both lungs,with a thin linear atelectatic band in the posterior segment of right lower pulmonary lobe and mildly thickened the pleura on the right side.
- Bilateral variable sized axillary lymphadenopathy,larger on the right side.
- Hepatomegaly with fatty infiltration.

3

Patient's Name: Samia Bakheet Habashi
Date: 2026-06-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Low lying malrotated right kidney, being located in the right lower lumbar region.
- An irregular mucosal thickening of the gastric wall, denoting a gastritis.
- No evidences of remote metastatic lesions seen in the present study.
- For clinical check up.

Prof/Hanan El-Ahwal.

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جوسناتفا
روستف
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
سجل

09/20/2026

MDT FORM

Name:	سليمه بنت حنن الجراح	MRN:	11357
Age:	53 Y	ID:	27307142401983
Menopausal Sta		Phone:*	01158674135
NRS SCORE:		Marital Status:	widow
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:	blood disease ???	POST OP	
Surgical History:*	hystrectomy	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX: ight breast: -Invasive tubular and cribriform carcinoma
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History:

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Mammo

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Recommendations:

ptn underwent upfront rt WLE+SLNB

Free margins

SLNB : 0/7

T1b, N0

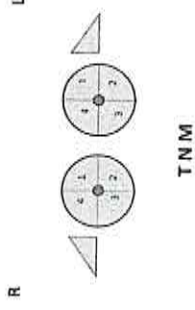
FOR adjuvant tt

MDT Outcome & Plan

for u/s contralat axilla

if free for adj TAM 5 years

PORT



Right Side

Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

Left Side

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د حنان عزت			

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جوسنافي
روسي
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
سجل

09/20/2026

MDT FORM

Name:	سليمه بنت حنن الراجحي	MRN:	11357
Age:	53 Y	ID:	27307142401983
Menopausal Sta		Phone:*	01158674135
NRS SCORE:		Marital Status:	widow
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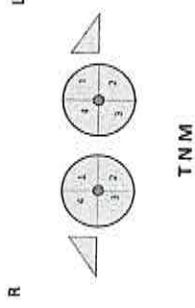
FOR adjuvant tt

MDT Outcome & Plan

for u/s contralat axilla

if free for adj TAM 5 years

PORT



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عزت			